NCCP 00361 · VERSION 4

Fulvestrant · injections every 4 weeks

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Fulvestrant: material risks

☐ Injection-site pain and uncommon nerve injury

☐ Joint / muscle pain

☐ Bleeding / bruising risk with intramuscular injections

☐ Hot flushes, fatigue and nausea

☐ Liver enzyme changes or liver injury

☐ Blood clots and hypersensitivity reactions

NCCP specifies 250 mg for Child-Pugh B, whereas EMA Faslodex product information describes no mild/moderate reduction with caution. The clinician and pharmacist must reconcile this source difference. Severe hepatic impairment is excluded. Where relevant, effective contraception is needed during treatment and for two years after the final dose under the EMA product information. Pregnancy and breastfeeding are excluded.

Individual benefit, alternatives and concerns

Agreed checks and next review

FBC, renal and liver profile. Confirm the indication, menopausal state, liver category, pregnancy exclusion and any bleeding / anticoagulant risk before intramuscular injections. Liver profile as clinically indicated. At injection visits review symptoms, bleeding / bruising and injection-site or nerve symptoms; response imaging follows the cancer-control plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.2 · Sources checked 2026-10-18
Source reconciled; independent clinical, pharmacy and patient/PPI review pending